

PATIENT HISTORY:

CHIEF COMPLAINT/PRE-OP/POST-OP DIAGNOSIS: Detrusor muscle invasive, high-grade urothelial carcinoma of the urinary bladder

PROCEDURE: Cystoprostatectomy and construction of a neobladder.

SPECIFIC CLINICAL QUESTION: Bladder cancer.

OUTSIDE TISSUE DIAGNOSIS: No.

PRIOR MALIGNANCY: No.

CHEMOTHERAPY: No.

ORGAN TRANSPLANT: No.

IMMUNOSUPPRESSION: No.

OTHER DISEASES: No.

1CB-0-3

Carcinoma urothelial 8/20/3

Site: bladder, lateral wall C67.2 pw 4/7/11

FINAL DIAGNOSIS:

Collection Date:

PART 1: URINARY BLADDER, DISTAL URETERS, PROSTATE, BILATERAL SEMINAL VESICLES, AND BILATERAL DISTAL VASA DEFERENTIA, RADICAL CYSTOPROSTATECTOMY -

FOR THE BLADDER:

- A. ULCERATED, INVASIVE UROTHELIAL CARCINOMA, HIGH-GRADE (2004 WHO/ISUP), PRESENT IN THE LEFT LATERAL URINARY BLADDER WALL (SLIDES 1K, 1L). CARCINOMA INVASES DEEPLY INTO, BUT NOT THROUGH THE DETRUSOR MUSCLE. CARCINOMA MEASURES APPROXIMATELY 1 CM IN MAXIMUM DIMENSION IN A HISTOLOGIC SECTION.
- B. EXTENSIVE, MULTIFOCAL UROTHELIAL CARCINOMA IN-SITU, INVOLVING RIGHT AND LEFT URETERAL OSTIA AND URETERS, ANTERIOR WALL, AND RIGHT AND LEFT LATERAL WALLS (SLIDES 1E-1G, 1I, 1J, 1M, 1O, 1P).
- C. ADJACENT FOCUS OF PAPILLARY UROTHELIAL CARCINOMA, HIGH-GRADE (2004 WHO/ISUP), INVOLVING THE POSTERIOR BLADDER WALL (SLIDES 1N, 1P).
- D. NO ANGIOLYMPHATIC INVASION IS IDENTIFIED.
- E. NO PERINEURAL INVASION IS IDENTIFIED.
- F. UROTHELIAL DYSPLASIA CLOSELY APPROACHES THE RIGHT DISTAL URETER RADIAL RESECTION MARGIN (SLIDE 1E). UROTHELIAL CARCINOMA IN-SITU CLOSELY APPROACHES THE CYSTOPROSTATECTOMY SPECIMEN LEFT DISTAL URETERAL RADIAL MARGIN; HOWEVER, A SEPARATE LEFT URETERAL MARGIN IS OBTAINED AND IS FREE OF UROTHELIAL NEOPLASIA (SEE PART 4). ALL OTHER SURGICAL RESECTION MARGINS ARE FREE OF IN-SITU / INVASIVE UROTHELIAL NEOPLASIA.
- G. NON-NEOPLASTIC URINARY BLADDER WITH CHRONIC CYSTITIS CYSTICA GLANDULARIS (SLIDE 1P) AND FOCAL GRANULOMATOUS INFLAMMATION (SLIDE 1K).
- H. TNM PATHOLOGIC STAGE (UROTHELIAL CARCINOMA): T2b N0 MX (See bladder tumor synoptic).

FOR THE PROSTATE:

- A. SINGLE SMALL FOCUS OF PROSTATIC ADENOCARCINOMA, ACINAR TYPE, GLEASON SCORE 3 + 3 = 6, LIMITED TO THE RIGHT LOBE OF THE PROSTATE GLAND (SLIDE 1R). PROSTATE GLAND SHOWS NO INVOLVEMENT BY THE UROTHELIAL CARCINOMA.
- B. MAXIMUM TUMOR DIAMETER OF THE PROSTATIC ADENOCARCINOMA IS 0.7 CM.
- C. PROSTATIC ADENOCARCINOMA INVOLVES LESS THAN 5% OF THE SAMPLED PROSTATE GLAND VOLUME.
- D. PROSTATIC ADENOCARCINOMA IS CONFINED TO THE PROSTATE GLAND, WITHOUT EXTRACAPSULAR EXTENSION NOR INVOLVEMENT OF THE ADJACENT BLADDER.
- E. NO PERINEURAL INVASION IS IDENTIFIED.
- F. NO ANGIOLYMPHATIC INVASION IS IDENTIFIED.
- G. FOCAL HIGH-GRADE PROSTATIC INTRAEPITHELIAL NEOPLASIA.
- H. BENIGN SEMINAL VESICLES AND DISTAL VASA DEFERENTIA.
- I. ALL SURGICAL RESECTION MARGINS ARE FREE OF PROSTATIC NEOPLASIA.
- J. TNM PATHOLOGIC STAGE (PROSTATIC ADENOCARCINOMA): T2a, N0, MX.
- K. HISTOLOGIC GRADE: G2.
- L. NON-NEOPLASTIC PROSTATE WITH NODULAR HYPERPLASIA (See prostate tumor synoptic).

PART 2: RIGHT OBTURATOR LYMPH NODE, EXCISION -
ONE BENIGN LYMPH NODE (0/1). NO EVIDENCE OF MALIGNANCY.

PART 3: LEFT PELVIC LYMPH NODES, EXCISION -
SIX BENIGN LYMPH NODES (0/6). NO EVIDENCE OF MALIGNANCY.

PART 4: LEFT DISTAL URETER, SHAVE MARGIN -

- A. COMPLETE CROSS-SECTION OF BENIGN URETER WITH FOCALLY DENUDED MUCOSA.
- B. NO EVIDENCE OF UROTHELIAL NEOPLASIA.

UUID: 4BCE7132-F235-4C57-9DAB-0C51C8737E87
TCGA-BT-A20W-01A-PR

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Criteria	Yes	No
Diagnosis Discrepancy		X
Primary Tumor Site Discrepancy		X
HIPAA Discrepancy		X
Prior Malignancy History	X	
Dual/Synchronous Primary, Noted		
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	MS	pw
Date Reviewed:	4/11/11	4/7/11

CASE SYNOPSIS:**SYNOPTIC DATA - PRIMARY URINARY BLADDER TUMORS**

SPECIMEN TYPE: Radical cystoprostatectomy
TUMOR SITE: Left lateral wall, Posterior wall
TUMOR SIZE: Greatest dimension: 1 cm
HISTOLOGIC TYPE: Urothelial (transitional cell) carcinoma
ASSOCIATED EPITHELIAL LESIONS: None identified
HISTOLOGIC GRADE: Urothelial carcinoma - High-grade
TUMOR CONFIGURATION: Papillary, Ulcerated
PATHOLOGIC STAGING (pTNM): pT2b
pN0

Number of nodes examined: 7

Number of nodes involved: 0

pMX

MARGINS: Margins uninvolved by invasive carcinoma

VENOUS/LYMPHATIC (LARGE/SMALL) VESSEL INVASION (V/L): Absent

DIRECT EXTENSION OF INVASIVE TUMOR: None identified

ADDITIONAL PATHOLOGIC FINDINGS: Urothelial dysplasia (low-grade intraurothelial neoplasia)

Inflammation/regenerative changes

Therapy-related changes

Comment:

Extensive urothelial carcinoma in-situ is present throughout the urinary bladder and ureters.

Urothelial dysplasia is close to the right ureteral resection margin.

SYNOPTIC DATA - PRIMARY PROSTATE TUMORS

INVASIVE CA IDENTIFIED?: Yes
TUMOR HISTOLOGY: Adenocarcinoma NOS
PRIMARY GLEASON GRADE: 3
SECONDARY GLEASON GRADE: 3
GLEASON SUM SCORE: 6
GLEASON 4/5 PERCENTAGE: 0%
WEIGHT OF PROSTATE: 100gm
TUMOR SIZE: Maximum dimension: 0.7 cm
LOBE LATERALITY: Right Lobe
PERCENT OF SPECIMEN INVOLVED BY TUMOR: < 5% of specimen involved by invasive tumor
MULTIFOCAL DISEASE: No
HIGH GRADE PIN: Yes - 1 or 2 foci away from tumor
EXTRAPROSTATIC EXTENSION: No
PERINEURAL INVASION: No
ANGIOLYMPHATIC INVASION: No
SEMINAL VESICLE INVASION: No
SURGICAL MARGIN INVOLVEMENT: All surgical margins free of tumor
LYMPH NODES EXAMINED: 7
LYMPH NODES POSITIVE: 0
T STAGE, PATHOLOGIC: pT2a
N STAGE, PATHOLOGIC: pN0
M STAGE, PATHOLOGIC: pMX
HISTOLOGIC GRADE: G2, Moderately differentiated

